NCCP 00284 · VERSION 5

Your treatment, explained

Locally advanced or metastatic NSCLC in older adults or people with ECOG2 · NCCP 00284 v5 · Document revised 4 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Gemcitabine alone · non-small-cell lung cancer

28-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call oncology immediately for fever or sudden illness, breathing difficulty, chest pain, uncontrolled vomiting, bleeding, very little urine, severe headache or confusion, or a spreading/blistering rash. Tell the nurse about infusion-site pain.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Open your CUH contact / treatment card

Sources: nccp

Choose a topic to read

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

- How each drug works
- Your overall treatment
- Your 28-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Tablets, visits and practical planning
- Supportive medicines
- Understanding the evidence
- Questions to ask
- Sources

UNDERSTAND YOUR TREATMENT

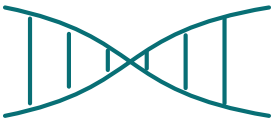
Your medicines

Gemcitabine aims to control lung cancer and its symptoms. Treatment is reviewed against side effects, your health and your priorities; benefit is not guaranteed.

CHEMOTHERAPY

Gemcitabine

Antimetabolite chemotherapy



Interferes with DNA building

Gemcitabine interferes with making DNA, which dividing cancer cells need. Healthy blood-forming cells and other tissues can also be affected.

DNA building → Replication disrupted → Dividing cells affected

What to watch for

Report fever, unusual bleeding, breathlessness, rash, swelling or reduced urine promptly.

Sources: nccp · smpc

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

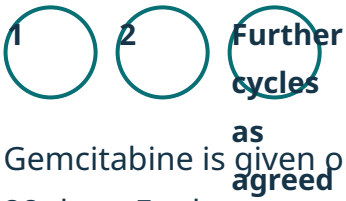
Explore your existing anatomy and side-effect map

THE BIGGER TREATMENT PLAN

Your overall treatment

3TREATMENT VISITS PER CYCLE

Repeated 28 day cycles



Gemcitabine is given on Days 1, 8 and 15. Week 4 has no planned dose. Each cycle lasts 28 days. For lung cancer there is no fixed six-cycle limit in this source: continue only while the team considers treatment helpful and tolerable.

NEXT PLAN AGREED

Review benefit and recovery

Review

Your team decides when to scan and whether to continue, pause or change treatment. Surgery and radiotherapy are separate plans.

Lung treatment duration is individual; the pancreatic six-cycle cap is not transferred here.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

My tests and checks

Required safety checks are distinct from individual response imaging and follow-up. No routine tumour-marker interval is specified.

Check	Before treatment	Treatment days	Between doses	Next cycle	After treatment
Treatment	Confirm lung indication	Gemcitabine Days 1/8/15	No dose Week 4	Repeat only if appropriate	Agree next plan
Blood tests	FBC, kidneys, liver	FBC before every dose	Extra tests if unwell	Renal/liver before cycle	Follow abnormalities
Other checks	Audiometry/CrCl if indicated	Review symptoms	Call when unwell	Review benefit and toxicity	Individual scans/ follow-up

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00284 v5 · official regimen

THE WHOLE TREATMENT JOURNEY

Gemcitabine alone · non-small-cell lung cancer: treatment and checks

Locally advanced or metastatic NSCLC in older adults or people with ECOG2

Gemcitabine is given on Days 1, 8 and 15. Week 4 has no planned dose. Each cycle lasts 28 days. For lung cancer there is no fixed six-cycle limit in this source: continue only while the team considers treatment helpful and tolerable.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

Treatment, tests and reviews across the course

Care / check	Before treatment	Treatment days	Between doses	Next cycle	After treatment
Treatment NCCP 00284	Confirm lung indication	Gemcitabine Days 1/8/15	No dose Week 4	Repeat only if appropriate	Agree next plan
Blood tests NCCP 00284	FBC, kidneys, liver	FBC before every dose	Extra tests if unwell	Renal/liver before cycle	Follow abnormalities
Other checks NCCP 00284	Audiometry/CrCl if indicated	Review symptoms	Call when unwell	Review benefit and toxicity	Individual scans/ follow-up

What each check is for

1

Prepare

Before first dose

Gemcitabine aims to control lung cancer and its symptoms. Treatment is reviewed against side effects, your health and your priorities; benefit is not guaranteed.

Ask / act: Confirm aim, prescription, tests and contacts.

Sources: NCCP 00284

2

Review

Before each dose

Check treatment remains appropriate

Ask / act: Report symptoms and confirm blood results before dosing.

Sources: NCCP 00284

3

Next plan

At agreed review points

Assess benefit and recovery

Ask / act: Confirm scan and follow-up dates with the team.

Sources: NCCP 00284

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-02 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.29

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Gemcitabine is given on Days 1, 8 and 15. Week 4 has no planned dose. Each cycle lasts 28 days. For lung cancer there is no fixed six-cycle limit in this source: continue only while the team considers treatment helpful and tolerable.



Timing varies: Tiredness, bowel changes and infection can occur between visits. Blood tests and your symptoms guide dose decisions; there is no guaranteed safe day.

Changes to the plan: The team may delay, omit or reduce doses for safety. Do not add a missed dose yourself.

Why these bands stay broad: Relative treatment days, without personal dates. Symptoms are not confined to a fixed day. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00284 v5 · official regimen ↗

There are no guaranteed "safe days". Call oncology immediately for fever or sudden illness, breathing difficulty, chest pain, uncontrolled vomiting, bleeding, very little urine, severe headache or confusion, or a spreading/blistering rash. Tell the nurse about infusion-site pain.

Read the whole cycle as text

Day 1: Gemcitabine only

Treatment after blood-test and symptom review. Your team confirms whether this dose can proceed.

Day 2–7: At home

No planned chemotherapy on these days. Use only the support medicines prescribed for today. Call using your contact plan if unwell; do not wait for a treatment day.

Day 8: Gemcitabine only

Treatment after blood-test and symptom review. Your team confirms whether this dose can proceed.

Day 9–14: At home

No planned chemotherapy on these days. Use only the support medicines prescribed for today. Call using your contact plan if unwell; do not wait for a treatment day.

Day 15: Gemcitabine only

Treatment after blood-test and symptom review. Your team confirms whether this dose can proceed.

Day 16–28: At home

No planned chemotherapy on these days. Use only the support medicines prescribed for today. Call using your contact plan if unwell; do not wait for a treatment day.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before each visit

Bring your medicine list and report illness. The team reviews blood results before deciding the dose.

STEP 2

Three treatment visits

Day 1,Day 8 and Day 15 are planned infusion visits. Week 4 has no routine dose.

STEP 3

At home

Keep the prescribed support medicines and urgent contact plan available. Do not make up a missed dose yourself.

Open practical treatment guide · Printable practical guide

Sources: NCCP 00284 v5 · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Sickness medicines

Gemcitabine has low sickness risk. Follow the prescribed preventive or rescue medicine plan; do not take a platinum anti-sickness course for gemcitabine alone.

2 Feeling unwell

Do not dismiss fever as a flu-like drug effect. Call the oncology team immediately for fever, shaking chills or sudden illness.

3 Medicines and fluids

Follow your individual drinking advice. Report reduced urine, swelling, breathlessness or trouble drinking, and ask before new medicines or supplements.

Sources: nccp · ncis

Understanding the evidence

Gemcitabine has been studied in older adults with advanced lung cancer.

Small historical single-arm older-adult study cannot establish a comparative survival benefit or predict an individual outcome.

Sources: ricci-2000

PREPARE FOR THE CONVERSATION

Questions worth asking

Why are there three visits?

The same medicine is planned on Days 1, 8 and 15, followed by a week without a planned dose. Each visit has its own safety checks.

Is treatment always six cycles?

No. That fixed duration belongs to the separate pancreatic adjuvant indication. Your lung treatment continues only while appropriate.

When will I have scans?

Your consultant agrees timing from your disease and treatment goals; this regimen does not mandate one interval.

> **Try explaining the plan back:** Explain back your treatment days, checks and whom to contact when unwell.

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00284 v5. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00284 v5 · official regimen
- Gemcitabine product information
- Ricci et al. 2000: gemcitabine in older adults with advanced NSCLC
- NCCP Antiemetic Medicines for inclusion in NCIS (Medical Oncology) · v8, 10 February 2025

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-02.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6699/284_v5_Gemcitabine_Monotherapy_28_day.pdf

https://www.medicines.org.uk/emc/product/3119/smpc

https://pubmed.ncbi.nlm.nih.gov/10688490/

https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf